



# ReconMed

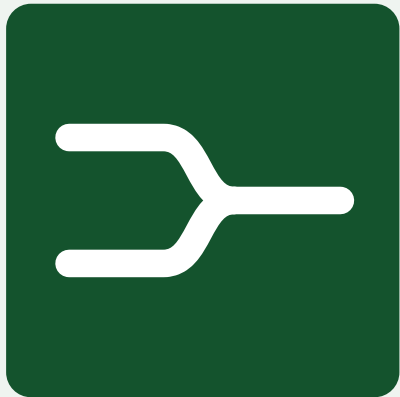
Automated Medicine Reconciliation

## Reconciled before the visit.

Pre-visit medication reconciliation  
for clinical trial sites.

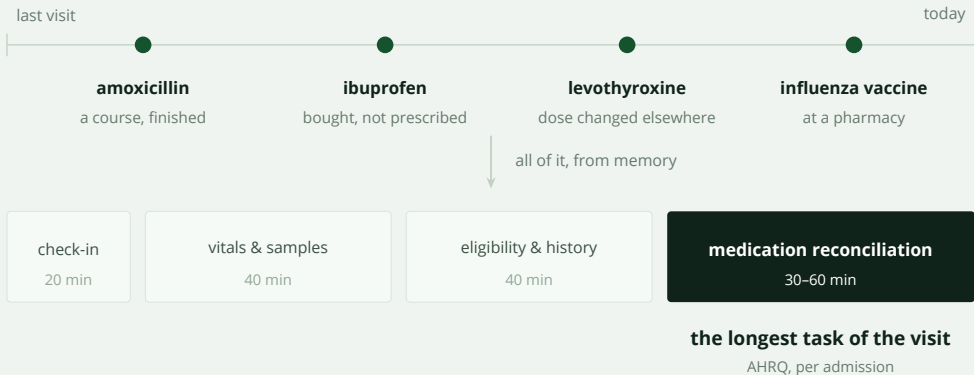
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HackMIT 2026



# The medication list is stale, and the visit is now

Every medication a participant takes besides the study drug is collected once, at the visit.



**Medication reconciliation happens at the visit, so the record is whatever the participant remembers.**

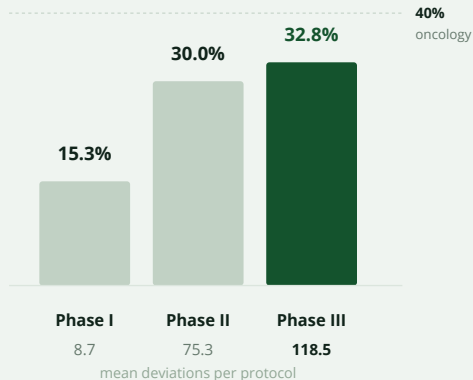
Nothing above reaches the site any other way. A course that ended six weeks ago is the easiest thing in the world to forget.

Reconciliation times: AHRQ *Patient Safety and Quality*; time-and-motion study of 103 reconciliations, *BMC Health Serv Res* 13:485. Hospital settings.

# Common in every trial, and expensive once it is cold

## Participants with a protocol deviation

187 protocols, 20 sponsors and CROs — Tufts CSDD, 2022



## Found later, in retrospective review

One phase 2 trial — I-SPY COVID, 2025

**\$4,862**

per data error identified

**\$6.1M**

and 61,073 person-hours, to check 30% of one trial

**0.36%**

of the fields checked actually held an error

# Calling participants between visits already works

25,829 pharmacist calls across 12 clinical trials — Martinez et al., 2017

## 45.5%

of those calls produced at least one intervention

Around 55% were rated high-impact — affecting trial outcomes or adherence.

### Participants with an issue found by phone

adverse events



medication use against protocol



concurrent medications



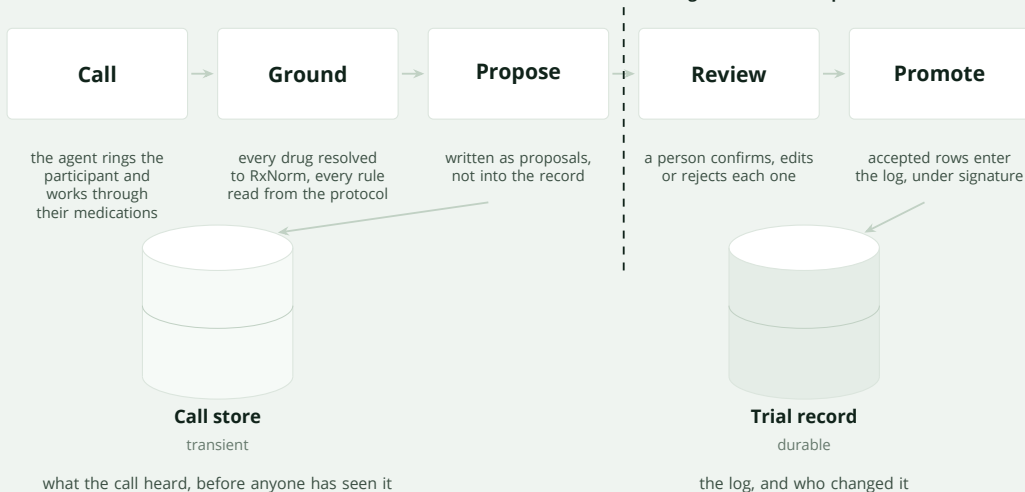
## A pharmacist made every one of those calls.

That is why it is not standard practice. ReconMed makes the call, and a coordinator reviews only what it found.

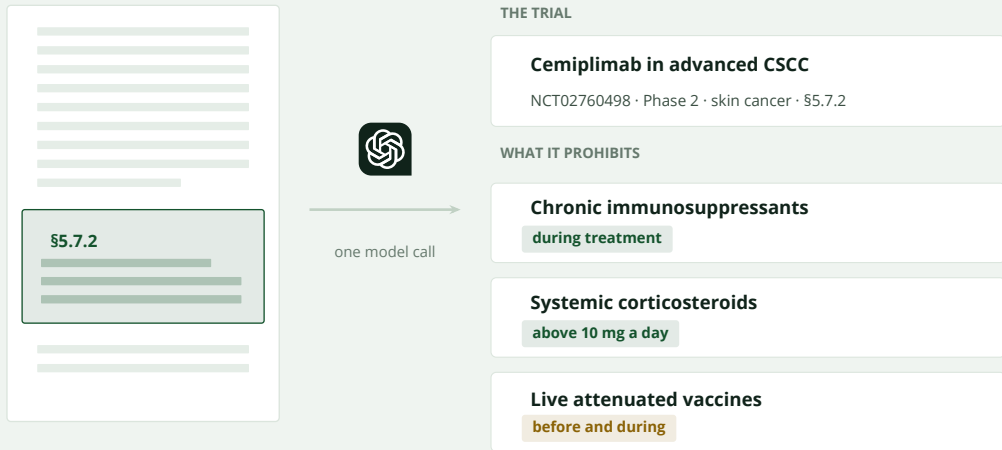
Martinez et al., *Clinical Therapeutics* 39(3), 2017. 3,873 participants; 12 trials in type 2 diabetes.

# How a call becomes a record

The agent proposes. A coordinator decides.



# Drop in the clinical trial protocol. It reads it.

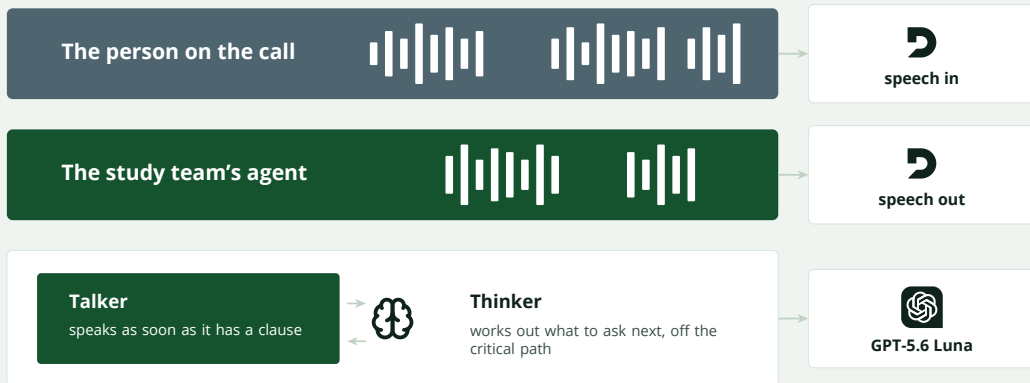


the published clinical protocol

a trial, ready to run

# Inside the voice agent

One model speaks. A second thinks between turns, so the line is never waiting.



**It may not answer from memory. Three questions, three sources:**

Is this drug banned here?

the trial's own protocol

What side effects are known?

the drug's FDA label

Is alcohol or smoking allowed?

the protocol's non-drug rules

# What we do about hallucination

The model is never the source. It may not state a fact it did not look up.



**GPT-5.6 Luna**

decides what to ask,  
never what is true



**RxNorm · RxClass**

what a drug is, and which class it belongs to



**the drug's FDA label**

which side effects are actually documented



**the participant's record**

the only source of anything about this person

heard

"prednasone"

resolved · RxCUI 8640

**prednisone**

classified

**ATC H02**

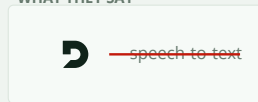
rule

**\$5.7.2**

# What we do about privacy

We took the participant's voice off the internet.

## WHAT THEY SAY

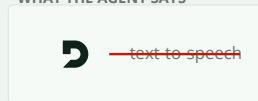


### Parakeet 0.6B

NVIDIA · open weights, on CPU

**Their recorded voice never reaches a vendor.**

## WHAT THE AGENT SAYS



### Qwen3-TTS 0.6B

Alibaba · open weights, on CPU

No API key, no upload, no account.  
The audio is transcribed and spoken  
on the site's own hardware.

## The record stays put too

The trial's medication log, the call in progress, the drug reference and the guidance all sit on the site's own machine.

## And the name never enters it

The call hears a name and a date of birth to verify identity. Both are stripped as it is filed, leaving the subject number.

The demo runs the hosted speech path for latency; the local models are built and tested, and the swap is a setting.



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Thank you · questions

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